California Rehabilitation

Fax Cover Sheet

Date: 4/1/2026

To: Two Trees Therapy & Wellness - Ortho Satellite Branch
5725 Ralston St. Suite 103
Ventura CA 93003
To Fax: 805-392-9975
To Phone: 805-360-5759

From: YAMEOGO, VICTORIEN, RN,
CALIFORNIA REHABILITATION INSTITUTE
2070 CENTURY PARK EAST
CENTURY CITY CA 90067
424-522-7100

Subject: Referral Request

Notice of Confidentiality

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PLEASE FIND REFERRAL FOR PATIENT DISCHARGING 04-02-26

Victorien
310-912-4018

California Rehabilitation

Aulakh, Navroj (MRN 27709)

Demographic Information

Legal Name: Aulakh, Navroj	Date of Birth: 01/17/1996	Legal Sex: Male
Preferred Name: Aulakh, Navroj	Age: 30 y.o.	Gender Identity: Male
Address: 3011 ELBAST APT 209 VENTURA CA 93003		
Home Phone: 224-440-3176	Mobile Phone:	Email:
Social Security Number: 624-90-0800	Race: Asian Indian	Ethnicity: Not Hispanic or Latino
Marital Status: Married	Religion: None	Language: English

Patient Account Information

Epic CSN	Account #	MRN	SM Corp ID
1027093633	37159	27709	2662725

Admission Information

Hospital Area	Department	Room/Bed	Service Area
California Rehabilitation Institute	CRI 2F COMP MED	241/241-A	REHAB SERVICE AREA

Attending Provider	Admitting Provider	Admission Type	Admission Status
Yang, Yue-Shan L, MD	Yang, Yue-Shan L, MD	Elective	Admitted

Admission Date/Time	Discharge Date/Time	Admission Diagnoses
03/18/2026 20:12		Pneumonia [J18.9]

Emergency Contact Information

Name	Relation	City	State	Zip	Home	Work
Alcantar,Leslie	Spouse				310-962-6469	
Sharma,Kapil	Mother				224-440-3177	

Additional Contact Information

	Relationship	Specialty	Notifications	Start	End
Generic External Data Provider				3/18/26	
Yue-Shan L Yang, MD	Physician	Physical Medicine and Rehabilitation		3/18/26	
Phone: 424-522-7100	Fax: 424-522-7099				
2070 Century Park East LOS ANGELES CA 90067-1907					

Insurance Information (Primary, Secondary, Tertiary)

Financial Class	Insurance Comments
Commercial	TRICARE WEST - Phone: (844)866-9378 - Grp: - Pol: 0156442800 - Auth: 0010229113

California Rehabilitation

California Rehabilitation Institute
2070 Century Park East
Century City CA 90067
Hosp Phone 424-522-7100
Hosp Fax 424-522-7099

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Comments

Expected Discharge Date and Time

Expected Discharge Date	Expected Discharge Time	Expected Discharge Time
Apr 2, 2026	10:00 AM	

Active Diet Orders
(From admission, onward)

Start		Ordered
03/21/26 1700	**Nutritional supplement Ensure Plus High Protein; 1 each; Oral** 2 times daily at breakfast and dinner	03/21/26 1122

Question	Answer	Comment
Select Supplement:	Ensure Plus High Protein	
Volume:	1 each	
Administration Route:	Oral	

03/18/26 2140	**Adult Diet Regular; 7 Regular (Regular Texture); 0 Thin (All Liquids)** Diet effective now	03/18/26 2139

Question	Answer	Comment
Diet Type:	Regular	
Diet Texture:	7 Regular (Regular Texture)	
Liquid Consistency:	0 Thin (All Liquids)	

PT Current Functional Status (Last filed) (since 3/2/2026)

	Value	Time	User
PT Current Functional Status	PT Weekly Note H&P (only for first weekly) 30 year old male with PMH of C3/C4 fusion who was transferred from OSH to UCLA Ronald Reagan for AHRF and septic shock 2/2 necrotizing PNA c/b ARDS on 3/7/26. He required ICU care. Hospital course notable for left vocal cord paresis resultingin dysphonia, Lue weakness likely from peripheral nerve injury, left sided muscle/flank strain, shock liver, nonischemic cardiomyopathy due to sepsis, acute renal failure, ams/icu delirium. Stable for CRI on 3/18/26. Week Summary/ Goals achieved/ progress Patient has established 3/3 STG this reporting period. Patient is making progress	3/31/2026 9:04 AM	Brian Bernal, PT

PT Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	in gait training and balance. Patient continues to be limited by LUE weakness and deconditioning. Goals for this week include balance training, and stair negotiation. Pt will continue to benefit from intensive inpatient rehab services to address the deficits listed above and to maximizing ease, safety, and independence with functional mobility. Precautions None. Prior Level of Function Home set up: Solo with 1 dog and 2 cats in an apt. Possible he may discharge to his parent's place in Pennsylvania Assistive Device used prior to admission: none Functional mobility: Patient was Indep with functional mobility. Patient was driving. Patient has 0 history of falls. Current Functional Level After Intervention (See QI's for baseline function upon Evaluation) -Bed mobility: Indep -Transfers: Indep -Gait: Indep -Stairs: Indep -Wheelchair mobility: Indep -Car transfer: Indep -Outcome measure: 2 minute step test, 112 steps total Clinical Presentation -Positive Prognostic factors: good LE strength -Limitations / Barriers: decreased single LE strength -Additional Information: c/s fusion Discharge Plan -LOS 15 days to achieve		

PT Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	Indep level for indoor mobility and Indep for community mobility. -D/C Plan (Location): home v parent's home -Support Post Discharge: none v family if he goes to his parent's place -Therapy follow-up: OPPT -DME Owned: none -DME required for discharge: none but TBD Prior to team conference, Brian Bernal, PT (member of primary therapy team) and Diana Kraikittikun, PT (designated therapist to attend team rounds) have communicated regarding current patient status, progress towards goals, and modifications to plan of care as appropriate. Medical chart reviewed in Epic prior to IDTC.		

OT Current Functional Status (Last filed) (since 3/2/2026)

	Value	Time	User
OT Current Functional Status	Week Summary/ Goals achieved/ Progress Patient has achieved all STG this reporting period. Patient is making progress in all self-care activities and functional mobility/transfers for ADLs. Patient continues to be limited by muscle weakness, vocal cord paralysis, impaired standing balance, impaired speech, impaired endurance, impaired sitting balance, stool impaction/constipation. Goals for this week include improving UB AROM above 90degrees against gravity improve wrist extension and complete ADLs with no pain. Pt will continue to benefit from intensive inpatient rehab services to address	3/31/2026 10:59 AM	Earl Gerald A. Rocha, OT

OT Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	the deficits listed above and to maximizing ease, safety, and independence with ADLs, IADLs, functional mobility, functional transfers, community re-integration, and participation in leisure activities.		
	H&P NAVROJ AULAKH is a 30 year old male diagnosed with Sepsis, unspecified organism and Pneumonia, unspecified organism. In an Inpatient Rehabilitation Facility, this patient will be classified under the Rehab Impairment Category of Miscellaneous. NAVROJ AULAKH requires an acute Inpatient Rehabilitation level of care to address his complex medical condition complicated by comorbidities which resulted in his functional impairments and increased risk for medical instability. Prior to referral for a rehabilitation admission, patient was Independent for household ambulation, Independent for community level ambulation, and Independent for self-care activities. Patient's past medical history and prior surgeries include: Past Medical History: ·No past medical history on file. Per HPI "C3/C4 fusion" Past Surgical History: ·SPINE SURGERY; 2024- C5 and C6 Patient's history of present illness includes: The patient is a 30 year old male with PMH of C3/C4 fusion who was transferred from OSH to UCLA Ronald Reagan for AHRF and septic shock 2/2 necrotizing PNA c/b ARDS. Hospital course notable for hypoxic respiratory failure s/p intubation then extubated, need for pressors/ critical		

OT Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	care stay, deconditioning, left sided muscle/flank strain, constipation, left vocal cord paresis and resulting dysphonia, LUE weakness post intubation, shock liver, non ischemic cardiomyopathy possibly due to sepsis, acute renal failure, AMS/ICU delirium, and over all decline in function. Prior to hospitalization the patient was an active independent community ambulator. Per current chart review of therapy documentation, and communication with nursing staff, they currently require CGA to moderate assist for ADL's, CGA- min A for mobility, intermittent cues for safety with all activities. Patient is anticipated to actively participate and significantly benefit from the program, and therefore demonstrates good potential to make meaningful gains to ensure a safe return home in the community, at a mod I-I level of function. Admission Dx: Acute Hypoxic Respiratory Failure due necrotizing PNA Precautions NA Prior Level of Function - Lives with: Alone - Entrance/home set u; p: 3 STE without rail, elevator access to apartment, ~12 stairs indoors - Bathroom set-up: tub shower, standard toilet - Assist required with ADLs: IND - Assist required with IADLs/home management: IND - Assist with mobility/device used: IND		

OT Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	Functional Level After Intervention 03/19/2026 (See QI's for baseline function upon Evaluation) - Eating: Set-up---->IND - Oral care: Sup---->IND - Toileting: Sup---->SUP - Bathing: Min A---->SUP - UB dressing: Min A---- >SUP - LB dressing: Min A---- >SUP - Footwear: Min A---->SUP - Rolling L&R: Sup---->SUP - Sit to Supine: Sup - Supine to Edge of Bed: Sup - Sit / Stand transfer: Sup - Bed / Chair Transfer: Sup (no device) - Toilet Transfer: Sup (no device) CLOF as of 3/31/26 per conversation with PT and chart review: - Eating: Set-up---->IND - Oral care: Sup---->IND - Toileting: Sup---->IND - Bathing: Min A---->IND - UB dressing: Min A---- >IND - LB dressing: Min A---- >IND - Footwear: Min A---->IND - Rolling L&R: Sup---->IND - Sit to Supine: IND - Supine to Edge of Bed: IND - Sit / Stand transfer: IND - Bed / Chair Transfer: IND (no device) - Toilet Transfer: IND (no device) Clinical Presentation - Positive Prognostic factors: Motivated, Family support, high PLOF - Impairments / Limitations / Barriers: muscle weakness, vocal cord paralysis, impaired standing balance, impaired speech, impaired endurance, impaired sitting balance, stool impaction/constipation - Skin assessment tab		

OT Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	completed. Yes Discharge Plan - LOS x 15 days to achieve IND level for ADLs, IND for transfers, and IND for IADLs - D/C Plan (Location): Home - Therapy follow-up: OP - Support Post Discharge: NA - DME Owned: NA - DME/AE required for discharge: NA		

SLP Current Functional Status (Last filed) (since 3/2/2026)

	Value	Time	User
SLP Current Functional Status	Week Summary/ Goals achieved/ progress: Pt is making good progress with SLP treatment this week, achieving 2/2 STGs. One new goal established. Treatment this week focused on vocal function exercises, review of education re: vocal hygiene strategies, use of diaphragmatic breathing, decreasing vocal strain, and use of different pitches. Pt reported increased lung capacity. Pt reported observing improvement in volume of voice. Pt benefits from min verbal cues to incorporate diaphragmatic breathing while completing vocal exercises. Pt has demonstrated improvements in pitch glides and voicing compared to last week. Precautions N/A Current Functional Level: - Voice: MIN Previous Weeks Functional Level: - Voice: MOD	3/30/2026 5:54 PM	Lauren A. Bruns, CCC-SLP

SLP Current Functional Status (Last filed) (since 3/2/2026) (continued)

	Value	Time	User
	Discharge Plan - LOS 14 days to achieve - Therapy follow-up: recommend ST services for voice and f/u at ENT clinic as an outpatient for left vocal cord paresis.		

Notes

H&P Notes

H&P by Yue-Shan L Yang, MD at 3/19/2026 7:11 AM documented on Admission (Current) from 3/18/2026 in California Rehabilitation Institute

Author:	Yue-Shan L Yang, MD	Author Type:	Physician	Filed:	3/19/2026 10:43 AM
Note Status:	Signed	Cosign:	Cosign Not Required	Date of Service:	3/19/2026 7:11 AM
Editor:	Yue-Shan L Yang, MD (Physician)				
Prior Versions:	1. Antonio Casco-Zuleta, MD (Resident) at 3/19/2026 9:57 AM - Cosign Needed				
	2. Yue-Shan L Yang, MD (Physician) at 3/19/2026 7:15 AM - Shared				

History and Physical

Patient Name: Navroj Aulakh
Admission Date and Time: 3/18/2026 8:12 PM
Room Number: 241/241-A

CC: decline in function s/p septic shock

History of present illness: 30 year old male with PMH of C3/C4 fusion who was transferred from OSH to UCLA Ronald Reagan for AHRF and septic shock 2/2 necrotizing PNA c/b ARDS on 3/7/26. He required ICU care. Hospital course notable for left vocal cord paresis resultingin dysphonia, Lue weakness likely from peripheral nerve injury, left sided muscle/flank strain, shock liver, nonischemic cardiomyopathy due to sepsis, acute renal failure, ams/icu delirium. Stable for CRI on 3/18/26.

Today, patient reports that he is doing well overall. States that he still has left upper extremity weakness that has been present since he woke up (after intubation), but that it has been improving and now he can make a fist with his left hand. Reports that he is unsure if voice is improving, but that he hopes so. States that he plans on going back to live with his parents in Pennsylvania, unless he is independent enough to got back to Port Hueneme where he is station, after rehab. States that he mostly does desk work, but that he does drive a motorcycle. Patient endorses ongoing left flank pain that does not radiate to his arm or elsewhere. Patient endorsing ongoing productive cough, especially after breathing treatments. However, reports that breathing has greatly improved overall. No other acute concerns at this time. He is happy to be at rehab.

Past Medical History:

Diagnosis	Date
• Lung disease	

No past surgical history on file.

H&P Notes (continued)

History reviewed. No pertinent family history.

Support System and Family Circumstances (i.e., potential caregivers):
Plans to discharge to his paren'ts home in pennsylvania

Prior Functional Status:
Premorbid Functional Status: independent, no device

Current Functional Status: Eating: supervision Grooming / Hygiene: min assist Upper Extremity Dressing: min assist Lower Extremity Dressing: moderate assistance Bed Mobility: supervision Supine-Sit: contact guard Sit-Stand: min assist Transfer: min assist Toilet Transfer: min assist Expression: independent Memory: independent Ambulation: min assist

Home Environment / Accessibility:
3 ste
12 stairs

Review of Systems:
As per HPI

Medications/Allergies:
Prescriptions Prior to Admission
No medications prior to admission.

Current Medications

Current Facility-Administered Medications:
• acetaminophen (TYLENOL) tablet 1,000 mg, 1,000 mg, Oral, Q8H PRN, Yue-Shan L Yang, MD, 1,000 mg at 03/18/26 2110
• albuterol (ACCUNEB) nebulizer solution 2.5 mg, 2.5 mg, Nebulization, RT BID, Yue-Shan L Yang, MD, 2.5 mg at 03/19/26 0752
• alum & mag hydroxide-simeth enteral suspension 30 mL, 30 mL, Oral, Q6H PRN, Yue-Shan L Yang, MD
• Baclofen (LIORESAL) tablet 5 mg, 5 mg, Oral, TID PRN, Yue-Shan L Yang, MD
• bisacodyl (DULCOLAX) suppository 10 mg, 10 mg, Rectal, Daily PRN, Yue-Shan L Yang, MD
• lidocaine (LIDOCARE) 4 % patch 1 patch, 1 patch, Transdermal, Once a day **AND** Check Patch Placement, , Transdermal, 2 times per day, Yue-Shan L Yang, MD, Check Patch Placement at 03/19/26 0805
• enoxaparin (LOVENOX) syringe 40 mg, 40 mg, Subcutaneous, Daily at 1800, Yue-Shan L Yang, MD
• guaiFENesin (MUCINEX) 12 hr tablet 600 mg, 600 mg, Oral, 3 times per day, Yue-Shan L Yang, MD, 600 mg at 03/19/26 0805
• melatonin tablet 5 mg, 5 mg, Oral, Nightly, Yue-Shan L Yang, MD, 5 mg at 03/18/26 2112
• naloxone (NARCAN) injection 0.1 mg, 0.1 mg, Intravenous, PRN, Yue-Shan L Yang, MD
• ondansetron ODT (ZOFRAN-ODT) disintegrating tablet 4 mg, 4 mg, Oral, Q6H PRN, Yue-Shan L Yang, MD
• oxyCODONE (ROXICODONE) immediate release tablet 2.5 mg, 2.5 mg, Oral, Q6H PRN, Yue-Shan L Yang, MD
• pantoprazole (PROTONIX) EC/DR tablet 40 mg, 40 mg, Oral, Once a day, Yue-Shan L Yang, MD, 40 mg at 03/19/26 0805
• polyethylene glycol (MIRALAX) packet 17 g, 17 g, Oral, 2 times per day, Yue-Shan L Yang, MD, 17 g at 03/19/26 0805
• polyethylene glycol (MIRALAX) packet 17 g, 17 g, Oral, Daily PRN, Yue-Shan L Yang, MD
• senna (SENOKOT) tablet 8.6 mg, 8.6 mg, Oral, Nightly, Yue-Shan L Yang, MD, 8.6 mg at 03/18/26 2112
• sodium chloride 3 % nebulizer solution 4 mL, 4 mL, Nebulization, RT BID, Yue-Shan L Yang, MD, 4 mL at 03/19/26 0752
• traZODone (DESYREL) split tablet 50 mg, 50 mg, Oral, Nightly PRN, Yue-Shan L Yang, MD, 50 mg at 03/18/26 2157

H&P Notes (continued)

Allergies
No Known Allergies

Vital Signs:

Vitals:	
	03/19/26 0812
BP:	
Pulse:	82
Resp:	16
Temp:	
SpO2:	99%

Ht./ Wt./ BMI:
Height: 5' 11" (180.3 cm) Weight: 168 lb 6.4 oz (76.4 kg) Body mass index is 22.73 kg/m².

Physical Exam:
Gen: NAD, pleasant, nontoxic. Sitting up in bed comfortably
HEENT: MMM, NCAT, EOMI, anicteric sclera
Neck: Supple, normal ROM
CV: RRR, no MRG
Lungs: CTAB
Abd: Soft, NT, ND, NABS
Skin: No rash
Extr: no edema, clubbing, cyanosis No joint contractures.
Psych: normal affect, behavior, thought processes.
Neuro:
Awake and alert
Oriented to person, place, date
Follows simple commands appropriately
Dysphonic speech
CN II-XII grossly intact
Diminished sensation to light touch in LUE. Otherwise, sensation to light touch grossly intact in all other extremities

	SA	EF	EE	WE	WF	FA	FF
Right	5	5	5	5	5	5	5
Left	4	4	4	3	4	3	4

	HF	KF	KE	DF	PF	EHL
Right	5	5	5	5	5	5
Left	5	5	5	5	5	5

Imaging:
Imaging studies reviewed.
CHEST X-RAY - 03/07/2026
Description/Location: CHEST X-RAY AT OSH
Results: FINDINGS: Lungs: Patchy airspace opacities involving the left upper lobe, lingula and likely lower lobe.
Cardiomediastinal silhouette: Unremarkable. Pleural margins: No effusion or pneumothorax. Soft tissues and

H&P Notes (continued)

bony
thorax: Metallic hardware projecting over the lower cervical spine. IMPRESSION: Multilobar left pneumonia.

US - 03/09/2026
Description/Location: US Abd 3/9/26
Results: 1. Mildly enlarged but otherwise normal appearing liver. 2. Incidental small left pleural effusion.

ASSESSMENT:
30 year old male with PMH of C3/C4 fusion who was transferred from OSH to UCLA Ronald Reagan for AHRF and septic shock 2/2 necrotizing PNA c/b ARDS.

Patient to be admitted to address functional deficits and for management of medical comorbidity related to septic shock, and for rehabilitation and medical issues related to AHRF. We will provide comprehensive inpatient rehabilitation, which may include PT, OT, RT, SLP, and supportive services to improve functional outcome of impaired mobility, ADLs, transfers, and self-care.

Patient requires 15 hours of therapy weekly for gait, balance, and ADL deficits that cannot be adequately treated at a lesser level of care. Patient requires 15 hours of therapy weekly for mobility, transfers, strength, and ADL deficits that cannot be adequately treated at a lesser level of care.

Code status has been reviewed with patient . Patient is a full code.

PLAN:
Active Problems:
Pneumonia

Rehab s/p Septic shock w AHRF
- Physical therapy for progressive range of motion, transfers, endurance/ balance, falls prevention, gait and stair training
- Occupational therapy for activities of daily living, adaptive equipment, functional transfers, upper extremities range of motion and strengthening exercises
- Speech Therapy for cognitive evaluation, communication/language evaluation, speech re-education, and swallow evaluation
- Consider Neuropsychology: adjustment, cognitive re-training, mood evaluation, neurocognitive testing
- Dietary for nutritional optimization
- Nursing: bladder/ bowel training, patient and family education, and skin/wound care.
- Case Manager to coordinate patient/family/caregiver conferences and discharge planning

#Acute hypoxic respiratory failure, resolved
#ARDS 2/2 Severe pneumonia with possible early necrotizing pneumonia, reolved
#Metabolic Alkalosis, resolved
Patient with CT findings c/f severe lingular pneumonia, intubated. Repeat CT chest showed dense consolidation throughout L lung and R lung base, with cystic changes in the left upper lobe containing patible with necrotizing pneumonia. Bronch on 03/07 completed, no mucous plugs or airway obstruction - thin yellow secretions seen. QUANT gold negative. Extubated as of 03/10 and on RA as of 03/11.
Dx:
- Bcx (03/07) x2 - NGTD
- Sputum cultures bacterial, fungal (03/07) - Candida likely colonizer
[x] MRSA nares negative
[x] Cocci IgG - negative
[x] Aspergillus - negative
[x] crypto - negative

H&P Notes (continued)

- Immunoglobulins wnl 03/08

Tx:

- s/p steroids: Hydrocort 50 mg IV q6h (03/07 - 03/11), s/p Solu-Medrol 40 mg IV BID (03/04 - 03/07 am dose)
- Abx:
    - s/p Azithromycin 500 mg IV q24h (03/05 - 3/9)
    - s/p Meropenem 1g q8h (03/07 - 3/13) for 7 day course total
    - s/p Vancomycin (03/05 - 03/09)
    - S/p Zosyn (03/05 - 03/07), Cefepime 2 g IV q12h (03/05), Meropenem 1g IV x1 (03/05), and CTX x1 (03/05)
    - S/p Tamiflu
- Airway clearance:
    - albuterol nebs BID
    - hypertonic saline BID, IPV
    - guaifenesin TID for 9 doses (3/18-3/21)

#Pleuritic chest pain, resolved

Likely 2/2 recovering PNA. EKG with no e/o ischemia/ACS

#Left sided pain (lower back and flank)

Tenderness in left lower paraspinal and flank areas. No CVA tenderness. Consistent with musculoskeletal strain vs spasm.

-Lidocaine patches
-Trial PRN baclofen
- Acetaminophen PRN
- Oxy PRN
- CTM

#Left Vocal Cord Paresis
#Dysphonia

New onset after extubation on 3/10. Voice is softer and more hoarse than would be typically expected after extubation. Underwent bedside scope with ENT that showed left vocal cord paresis.

-ENT consulted at UCLA, appreciate recs.
    -Not a good candidate for vocal cord injection given strong gag reflex. Continue to monitor for now.
    -If no improvement, can follow with ENT clinic as an outpatient. Will provide contact information on discharge.
- SLP as above

#Left arm weakness, onset after extubation
#H/o C3-4 fusion

Low concern for stroke. No radicular symptoms. With distal weakness on grip strength and wrist flexion/extension. Appears consistent with peripheral neuropathy. Note has been in bed for prolonged time and proned on ventilator.

-Continue to monitor
-PT/OT as above
-Consider further evaluation as outpatient if no improvement, including EDX studies

#Hypokalemia, resolved

With U waves on EKG 3/12. Not on any QT prolonging meds.

-Replete K to 4 PRN
- F/u admission labs

#Leukocytosis, improved.

Received steroids, and recovering from infx

- Continue to monitor

Nonischemic cardiomyopathy, query septic , recovered EF
Mitral valve sclerosis, aortic valve thick sclerosis
Initial echo with EF of 25%, diffuse hypokinesis, grade 3 diastolic dysfunction, possible McConnell sign. Mitral valve sclerosis without stenosis.
[x] Repeat TTE - EF 62.5%, < 50% respiratory change although pt was intubated at this time.

#Shock liver, resolving
#Mild hepatomegaly
LFTs on arrival elevated. WNL 3/18 prior to discharge from UCLA
- Abd US - mildly enlarged liver, incidental pleural effusion
- Vitamin K 5mg once 03/07
- CTM

#Acute renal failure, resolved
Noted to have had acute renal failure at OSH, but recovered. Cr 0.71 at UCLA 3/18 prior to discharge
- F/u admission labs

#Acute illness/ICU delirium, resolved
No longer having visual hallucinations
- melatonin at bedtime
- trazodone 50 at bedtime prn

#Constipation
#RUQ tenderness, resolved
Previously endorsed RUQ pain in addition; however now resolved and RUQUS 3/17 wnl. LFTs stable.
- Miralax BID
- Senna 1 tab every night
- Bisacodyl Supp daily PRN

Nausea
- Ondansetron 4 mg q6h PRN

DVT Ppx- Enoxaparin 40 mg SubQ daily

Code Status: Full Code

Goals: mod I

Prognosis: At the current time, this inpatient hospital rehabilitation stay is medically necessary to achieve important health and functional goals. The patient requires frequent physician visits, 24-hour rehabilitation nursing, and a coordinated intensive rehabilitation program as described above to address complex medical, nursing, and rehabilitation needs. The patient has a fair prognosis for benefiting from this program and returning home and community.

Estimated Length of Stay: 10-14 days

Reviewed case and discussed care plan with attending physician

CASCO-ZULETA, ANTONIO, MD
3/19/2026

H&P Notes (continued)

9:57 AM PDT

I have reviewed the patient's history and I have personally examined the patient. I agree with the findings and plan of care documented in the above resident's note, with any additions or modifications noted below and/or edited in note.

I spent 76 minutes today involved in both hospital chart review and direct patient care/medical management, including face-to-face and unit/floor time dedicated to addressing the specifics of the patient's medical and rehabilitation-related issues.

YANG, YUE-SHAN L, MD
Attending Physician,
California Rehabilitation Institute
3/19/2026
10:42 AM PDT

Physician Progress Notes (last 24 hours) (Notes for yesterday and today)

Progress Notes by Yue-Shan L Yang, MD at 3/31/2026 11:25 AM Version 1 of 1

Author: Yue-Shan L Yang, MD | Service: Physical Medicine and Rehabilitation | Author Type: Physician
Filed: 3/31/2026 1:14 PM | Date of Service: 3/31/2026 11:25 AM | Status: Signed
Editor: Yue-Shan L Yang, MD (Physician)

Rehab Progress Note

CC: decline in function s/p septic shock

Subjective:
No acute events overnight
PRN Oxy 2.5 x1
Refusing lovenox.[YY.1]
Left hand and arm pain worse. Strength improving.[YY.2]

Review of Systems:
As above. Otherwise negative

Functional Assessment:
40 stairs

Objective:
Vital signs:
Last 24 hours: Temp: [97.4 °F (36.3 °C)-97.9 °F (36.6 °C)] 97.4 °F (36.3 °C)
Pulse: [89-110] 100
Resp: [18] 18
BP: (126-128)/(76-77) 126/77
FiO2 (%): [21 %] 21 %

Weight:

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Yue-Shan L Yang, MD at 3/31/2026 11:25 AM (continued) Version 1 of 1

Admit Weight: 168 lb 6.4 oz (76.4 kg)
Latest Weight: 170 lb (77.1 kg)

Scheduled Meds:
Scheduled/Continuous Medications

Scheduled

Check Patch Placement (And Linked Group #1)
No Dose/Rate, TD, BID
enoxaparin (LOVENOX) syringe 40 mg
40 mg, SC, Daily at 1800
gabapentin (NEURONTIN) capsule 300 mg
300 mg, PO, TID
ipratropium-albuterol (DUO-NEB) 0.5-2.5 mg/3 mL nebulizer solution 3 mL
3 mL, IN, RT BID
lidocaine (LIDOCARE) 4 % patch 1 patch (And Linked Group #1)
1 patch, TD, Once a day
melatonin tablet 5 mg
5 mg, PO, Nightly
pantoprazole (PROTONIX) EC/DR tablet 40 mg
40 mg, PO, Once a day
polyethylene glycol (MIRALAX) packet 17 g
17 g, PO, BID
senna (SENOKOT) tablet 8.6 mg
8.6 mg, PO, Nightly
sodium chloride 3 % nebulizer solution 4 mL
4 mL, NEBULIZATION, RT BID

Continuous Infusions:
PRN Meds:.acetaminophen, 1,000 mg, Oral, Q8H PRN
Alum & Mag Hydroxide-Simeth, 30 mL, Oral, Q6H PRN
Baclofen, 5 mg, Oral, TID PRN
bisacodyl, 10 mg, Rectal, Daily PRN
naloxone (NARCAN) injection, 0.1 mg, Intravenous, PRN
ondansetron ODT, 4 mg, Oral, Q6H PRN
oxyCODONE, 2.5 mg, Oral, Q6H PRN
polyethylene glycol, 17 g, Oral, Daily PRN
traZODone, 50 mg, Oral, Nightly PRN

Physical Exam:
Gen: NAD, pleasant, nontoxic. Ambulating independently
HEENT: MMM, NCAT, EOMI, anicteric sclera
Neck: Supple, normal ROM

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Yue-Shan L Yang, MD at 3/31/2026 11:25 AM (continued) Version 1 of 1

CV: RR
Lungs: breathing comfortably on room air. CTAB
Back: Mildly ttp of left flank/low back. No ecchymosis/rash appreciated
Skin: No rash
Extr: no edema, clubbing, cyanosis No joint contractures.
Psych: normal affect, behavior, thought processes.
Neuro:
Awake, alert, and oriented
Follows simple commands appropriately
Dysphonic speech
CN II-XII grossly intact
Diminished sensation to light touch in LUE. Otherwise, sensation to light touch grossly intact in all other extremities[YY.1]
4/5 left hand finger abduction, 4/5 left finger flexion, 3/5 left wrist extension, and left wrist flexion[YY.2]

Lab Results:
I have personally reviewed the following labs / studies:

Lab Results

Component	Value	Date
WBC	7.91	03/30/2026
HGB	12.7 (L)	03/30/2026
HCT	39.3	03/30/2026
MCV	84.5	03/30/2026
PLT	345	03/30/2026

Lab Results

Component	Value	Date
GLUCOSE	89	03/30/2026
CALCIUM	9.2	03/30/2026
NA	137	03/30/2026
K	4.2	03/30/2026
CO2	25	03/30/2026
CL	102	03/30/2026
BUN	12	03/30/2026
CREATININE	0.82	03/30/2026

CT Chest 3/28/26
Impression:
Consolidative changes are seen within the left upper lobe apicoposterior segment. There are prominent lucencies within the consolidative focus which may reflect foci of cavitation and/or bronchiectasis and bronchiolectasis. Bronchoscopic correlation is
recommended.

ASSESSMENT:
30 yo M with PMH ff C3/C4 fusion, recently hospitalized due to AHRF and septic shock 2/2 necrotizing PNA c/b ARDS.

Active Problems:
Pneumonia

PLAN:

Rehab s/p Septic shock w AHRF

- Physical therapy for progressive range of motion, transfers, endurance/ balance, falls prevention, gait and stair training
- Occupational therapy for activities of daily living, adaptive equipment, functional transfers, upper extremities range of motion and strengthening exercises
- Speech Therapy for cognitive evaluation, communication/language evaluation, speech re-education, and swallow evaluation
- Consider Neuropsychology: adjustment, cognitive re-training, mood evaluation, neurocognitive testing
- Dietary for nutritional optimization
- Nursing: bladder/ bowel training, patient and family education, and skin/wound care.
- Case Manager to coordinate patient/family/caregiver conferences and discharge planning

#Acute hypoxic respiratory failure, resolved
#ARDS 2/2 Severe pneumonia with possible early necrotizing pneumonia, reolved
#Metabolic Alkalosis, resolved

Patient with CT findings c/f severe lingular pneumonia, intubated. Repeat CT chest showed dense consolidation throughout L lung and R lung base, with cystic changes in the left upper lobe containing patible with necrotizing pneumonia. Bronch on 03/07 completed, no mucous plugs or airway obstruction - thin yellow secretions seen. QUANT gold negative. Extubated as of 03/10 and on RA as of 03/11.

Dx:
- Bcx (03/07) x2 - NGTD
- Sputum cultures bacterial, fungal (03/07) - Candida likely colonizer

[x] MRSA nares negative
[x] Cocci IgG - negative
[x] Aspergillus - negative
[x] crypto - negative

- Immunoglobulins wnl 03/08

CXR 3/25 showing large consolidation and PNA in periphery of L lung
-Discussed with IM. CXR improved compared to prior.
CT Chest 3/28 with consolidative changes seen within the left upper lobe
-F/u w pulm as outpatient, consider bronchoscopy

Tx:
- s/p steroids: Hydrocort 50 mg IV q6h (03/07 - 03/11), s/p Solu-Medrol 40 mg IV BID (03/04 - 03/07 am dose)
- Abx:
  - s/p Azithromycin 500 mg IV q24h (03/05 - 3/9)
  - s/p Meropenem 1g q8h (03/07 - 3/13) for 7 day course total
  - s/p Vancomycin (03/05 - 03/09)
  - S/p Zosyn (03/05 - 03/07), Cefepime 2 g IV q12h (03/05), Meropenem 1g IV x1 (03/05), and CTX x1 (03/05)
  - S/p Tamiflu
- Airway clearance:
  - Duo-nebs BID
  - hypertonic saline BID, IPV
  - guaifenesin TID for 9 doses (3/18-3/21, completed)
- Pulm consulted. Appreciate recs

#Pleuritic chest pain, resolved

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Yue-Shan L Yang, MD at 3/31/2026 11:25 AM (continued) Version 1 of 1

Likely 2/2 recovering PNA. EKG with no e/o ischemia/ACS

#Left sided pain (lower back and flank)
Tenderness in left lower paraspinal and flank areas. No CVA tenderness. Consistent with musculoskeletal strain vs spasm.
-Lidocaine patches
-Trial PRN baclofen
- Acetaminophen PRN
- Oxy PRN
- CTM

#Left Vocal Cord Paresis
#Dysphonia
New onset after extubation on 3/10. Voice is softer and more hoarse than would be typically expected after extubation. Underwent bedside scope with ENT that showed left vocal cord paresis.
-ENT consulted at UCLA, appreciate recs.
    -Not a good candidate for vocal cord injection given strong gag reflex. Continue to monitor for now.
    -If no improvement, can follow with ENT clinic as an outpatient. Will provide contact information on discharge.
- SLP as above

#Left arm weakness, onset after extubation
#H/o C3-4 fusion
#LUE Neuropathic pain
Low concern for stroke. No radicular symptoms. With improving distal weakness on grip strength and wrist flexion/extension. Note has been in bed for prolonged time and proned on ventilator.
-Continue to monitor
-PT/OT as above
- Gabapentin increased to[YY.1] 4[YY.2]00 TID (3/[YY.1]31[YY.2])
    - Uptitrate as appropriate
-Consider further evaluation as outpatient if no improvement, including EDX studies

#Hypokalemia, resolved
With U waves on EKG 3/12. Not on any QT prolonging meds.
K- 4.0 on admission labs (3/19)
-Replete K to 4 PRN
- CTM

#Leukocytosis, resolved
Received steroids, and recovering from infx
WBC- 7.09 on admission labs (3/19)
- Continue to monitor

Nonischemic cardiomyopathy, query septic , recovered EF
Mitral valve sclerosis, aortic valve thick sclerosis
Initial echo with EF of 25%, diffuse hypokinesis, grade 3 diastolic dysfunction, possible McConnell sign. Mitral valve sclerosis without stenosis.
[x] Repeat TTE - EF 62.5%, < 50% respiratory change although pt was intubated at this time.

#Shock liver, resolving
#Mild hepatomegaly

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Yue-Shan L Yang, MD at 3/31/2026 11:25 AM (continued) Version 1 of 1

LFTs on arrival elevated. WNL 3/18 prior to discharge from UCLA
- Abd US - mildly enlarged liver, incidental pleural effusion
- Vitamin K 5mg once 03/07
- CTM

#Acute renal failure, resolved
#Hyponatremia
Noted to have had acute renal failure at OSH, but recovered. Cr 0.71 at UCLA 3/18 prior to discharge
Na- 133 on admission labs
- CTM

#Acute illness/ICU delirium, resolved
No longer having visual hallucinations
- melatonin at bedtime
- trazodone 50 at bedtime prn

#Constipation
#RUQ tenderness, resolved
Previously endorsed RUQ pain in addition; however now resolved and RUQUS 3/17 wnl. LFTs stable.
- Miralax BID
- Senna 1 tab every night
- Bisacodyl Supp daily PRN

Nausea
- Ondansetron 4 mg q6h PRN

DVT Ppx- Enoxaparin 40 mg SubQ daily[YY.1], refusing and ambulating, will dc[YY.2]

The patients care plan was reviewed. Assessment and plan discussed and agreed to by treatment team members.

At the current time, this inpatient hospital rehabilitation stay is medically necessary to achieve important health and functional goals. The patient requires frequent physician visits, 24-hour rehabilitation nursing, and a coordinated intensive rehabilitation program as described above to address complex medical, nursing, and rehabilitation needs. The patient has a good prognosis for benefiting from this program and returning home and community.

Reviewed education relevant to patient treatments with patient/ family.
Performing a medically appropriate exam and evaluation, counseling and educating a patient/family/caregiver, placing relevant orders, and referring and communicating with other healthcare professionals.

Time of note does not reflect time of encounter.

YANG, YUE-SHAN L, MD
03/31/26
11:25 AM PDT[YY.1]

Electronically signed by Yue-Shan L Yang, MD on 3/31/2026 1:14 PM

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Yue-Shan L Yang, MD at 3/31/2026 11:25 AM (continued) Version 1 of 1

Attribution Key

YY.1 - Yue-Shan L Yang, MD on 3/31/2026 11:25 AM
YY.2 - Yue-Shan L Yang, MD on 3/31/2026 1:12 PM

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM Version 1 of 1

Author: Mary Henary, MD | Service: Internal Medicine | Author Type: Physician
Filed: 3/31/2026 11:34 AM | Date of Service: 3/31/2026 11:31 AM | Status: Signed
Editor: Mary Henary, MD (Physician)

California Rehabilitation Institute
A Cedars/UCLA Health Partnership

INTERNAL MEDICINE PROGRESS NOTE

UCLA HOSPITALIST SERVICE

UCLA Health

DATE OF SERVICE: 03/31/26
ADMISSION DATE: 3/18/2026

REQUESTING PHYSICIAN(S): Yue-Shan L Yang, MD

PMD: No primary care provider on file.

CHIEF COMPLAINT: weakness

REASON FOR CONSULTATION: Medical comanagement

INTERVAL EVENTS AND REVIEW OF SYSTEMS:
NAEON, VSS, remains on RA
Required 2.5mg oxycodone last night for ongoing pain to LUE. States sharp pain around median nerve with burning pain down the arm. Discussed increasing gabapentin for likely compressive neuropathy, patient in agreement
No CP, SOB, cough, N/V
Feels well otherwise
FU appts requested

Further review of systems negative except as noted above. Patient denies n/v, diarrhea, constipation, SOB, cough, chest pain, and palpitations.

MEDICATIONS:
Scheduled Meds:
Scheduled/Continuous Medications

Scheduled

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM (continued) Version 1 of 1

Check Patch Placement (And Linked Group #1)
No Dose/Rate, TD, BID
enoxaparin (LOVENOX) syringe 40 mg
40 mg, SC, Daily at 1800
gabapentin (NEURONTIN) capsule 400 mg
400 mg, PO, TID
ipratropium-albuterol (DUO-NEB) 0.5-2.5 mg/3 mL nebulizer solution 3 mL
3 mL, IN, RT BID
lidocaine (LIDOCARE) 4 % patch 1 patch (And Linked Group #1)
1 patch, TD, Once a day
melatonin tablet 5 mg
5 mg, PO, Nightly
pantoprazole (PROTONIX) EC/DR tablet 40 mg
40 mg, PO, Once a day
polyethylene glycol (MIRALAX) packet 17 g
17 g, PO, BID
senna (SENOKOT) tablet 8.6 mg
8.6 mg, PO, Nightly
sodium chloride 3 % nebulizer solution 4 mL
4 mL, NEBULIZATION, RT BID

Continuous Infusions:
PRN Meds:.• acetaminophen
• Alum & Mag Hydroxide-Simeth
• Baclofen
• bisacodyl
• naloxone (NARCAN) injection
• ondansetron ODT
• oxyCODONE
• polyethylene glycol
• traZODone

ALLERGIES:
Allergies
No Known Allergies[MH.1]

VITALS:
Temp: [97.4 °F (36.3 °C)-97.9 °F (36.6 °C)] 97.4 °F (36.3 °C)
Pulse: [89-110] 100
Resp: [18] 18
BP: (126-128)/(76-77) 126/77
FiO2 (%): [21 %] 21 %

BP 126/77 | Pulse 100 | Temp 97.4 °F (36.3 °C) (Oral) | Resp 18 | Ht 5' 11" (1.803 m) | Wt 170 lb (77.1 kg) | SpO2 98% | BMI 23.71 kg/m²

PHYSICAL EXAM:
General: No acute distress. Comfortable in therapy
CV: 2+ radial pulses, regular, normal rate

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM (continued) Version 1 of 1

Resp: Unlabored breathing. Symmetrical expansion.
Abdomen: Soft, non distended
Extremities: WWP, no edema

CHART REVIEW:
I have reviewed the relevant interval records since the last hospitalist visit, including laboratory results, imaging results, physician notes, and any available outpatient progress notes. I spoke with PMR and Nursing on 3/31/2026 about the case.

LABS:
BMP
Recent Labs

Lab	Units	03/30/26 0600
SODIUM	mmol/L	137
POTASSIUM	mmol/L	4.2
CHLORIDE	mmol/L	102
CO2	mmol/L	25
BUN	mg/dL	12
CREATININE	mg/dL	0.82
GLUCOSE	mg/dL	89
CALCIUM	mg/dL	9.2

LFT
Recent Labs

Lab	Units	03/30/26 0600
BILIRUBIN TOTAL	mg/dL	0.3
ALT	U/L	44
AST	U/L	27

CBC
Recent Labs

Lab	Units	03/30/26 0600
WBC AUTO	x10E3/uL	7.91
HEMOGLOBIN	g/dL	12.7*
HEMATOCRIT	%	39.3
MCV	fL	84.5
PLATELETS AUTO	x10E3/uL	345

Coags

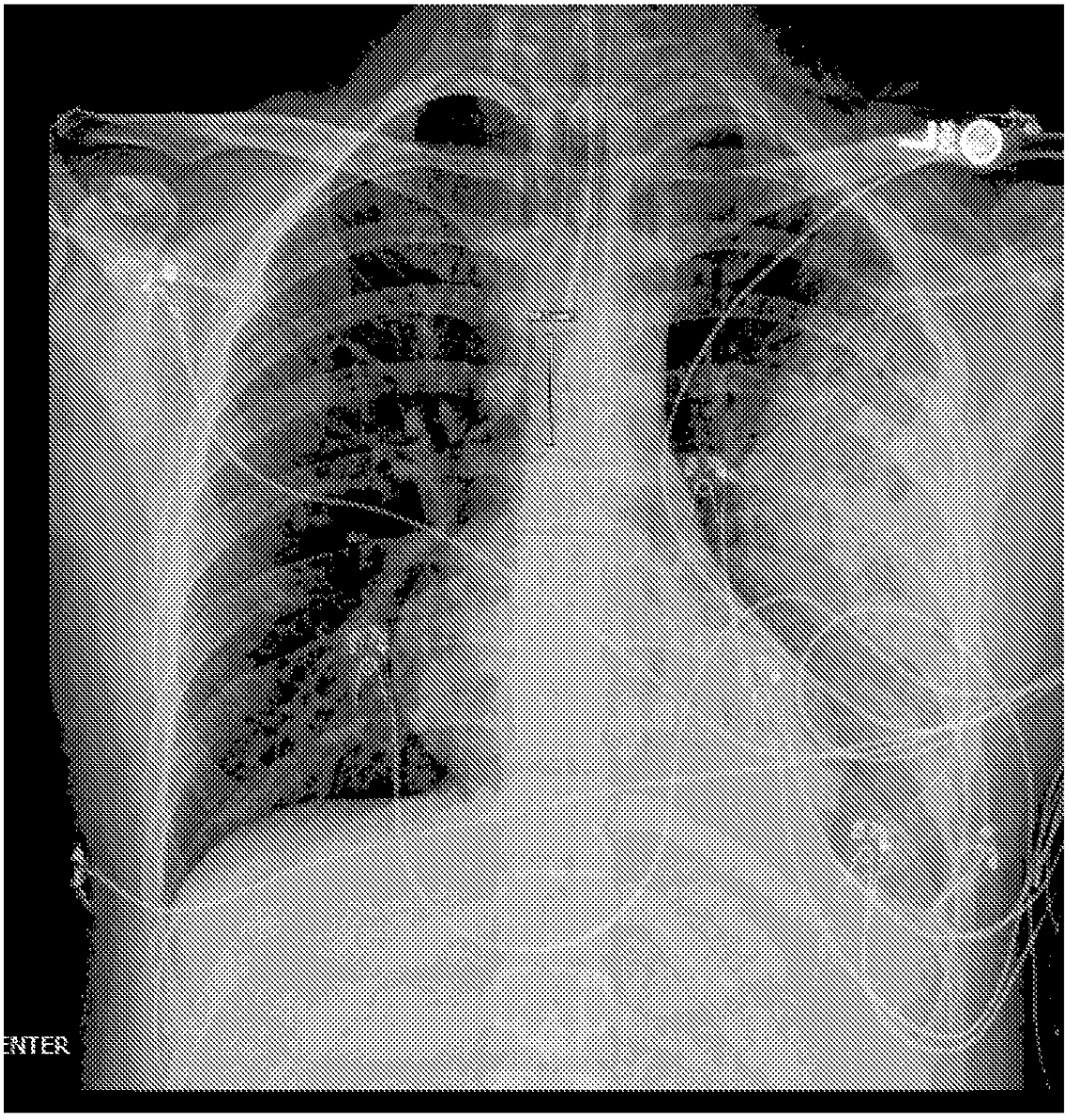
Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM (continued) Version 1 of 1

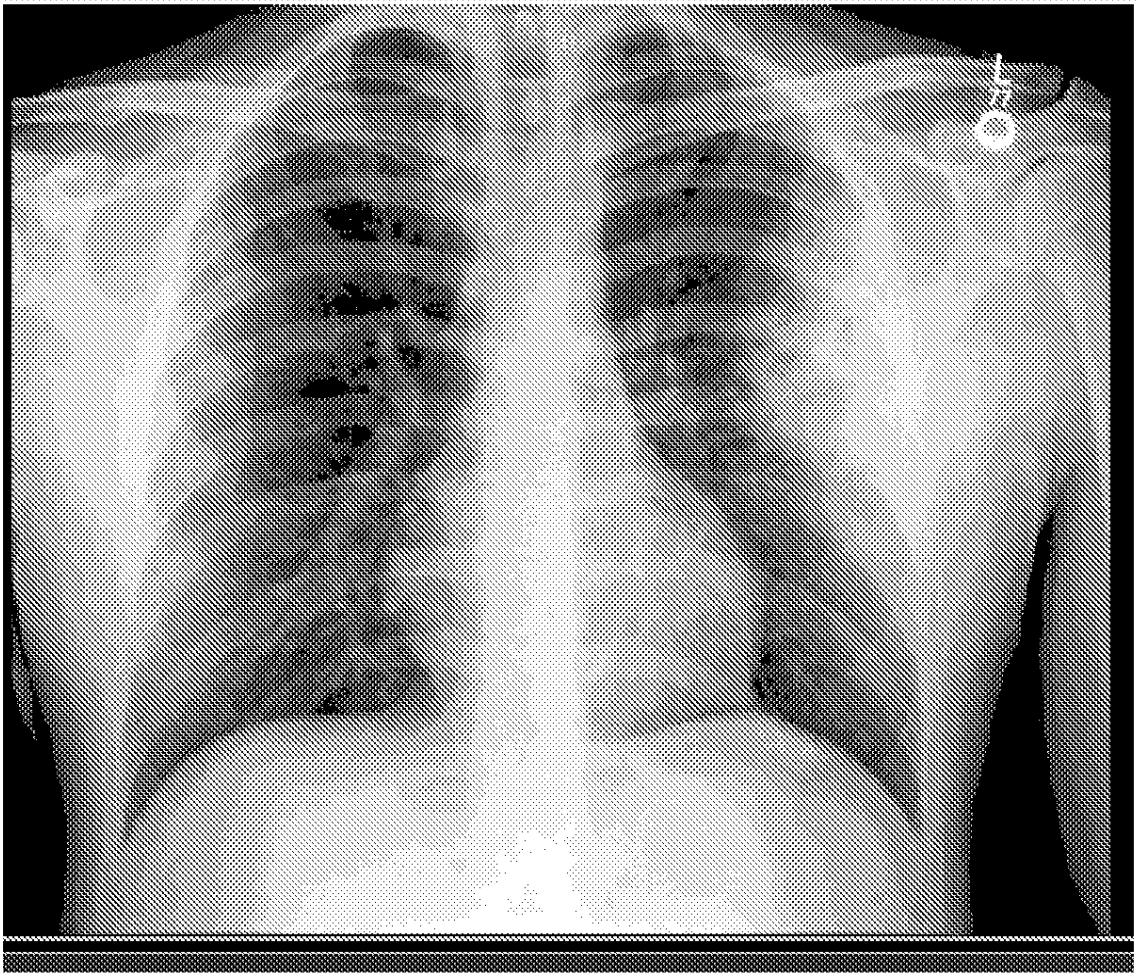
IMAGING:

3/26 CT chest non con
Consolidative changes are seen within the left upper lobe apicoposterior segment. There are prominent lucencies within the consolidative focus which may reflect foci of cavitation and/or bronchiectasis and bronchiolectasis. Bronchoscopic correlation is
recommended.

3/8/26 CXR at UCLA



3/25/26 CXR at CRI



STUDIES:

ASSESSMENT AND PLAN/RECOMMENDATIONS:
Navroj Aulakh is a 30 y.o. year old male with

#Deconditioning
-Continue PT/OT

#Left sided pain (lower back and flank) - improving
Tenderness in left lower paraspinal and flank areas. No CVA tenderness. Consistent with musculoskeletal strain vs spasm.
-Lidocaine patches
-Trial PRN baclofen
-Continue acetaminophen PRN and oxy PRN
-Continue PT/OT

#Constipation
#RUQ tenderness, resolved
Previously endorsed RUQ pain in addition; however now resolved and RUQUS 3/17 wnl. LFTs stable. Has been having BMs but reports that they have been small.
-Miralax BID

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM (continued) Version 1 of 1

-Senna 1 tab every night

#Left Vocal Cord Paresis
#Dysphonia
New onset after extubation on 3/10. Voice is softer and more hoarse than would be typically expected after extubation. Underwent bedside scope with ENT that showed left vocal cord paresis.
- ENT FU OP

#Left arm weakness, onset after extubation
#H/o C3-4 fusion
#Worsening Neuropathy, improving strength
Low concern for stroke. No radicular symptoms. With distal weakness on grip strength and wrist flexion/extension. Appears consistent with peripheral neuropathy. Note has been in bed for prolonged time and proned on ventilator.
3/31: still with ongoing compressive neuropathy. Increase gabapentin to 400mg TID
-Continue to monitor
-PT/OT as above
-Consider further evaluation OP with EMG

#Acute hypoxic respiratory failure, resolved
#ARDS 2/2 Severe pneumonia with possible early necrotizing pneumonia, reolved
#Metabolic Alkalosis, resolved
Patient with CT findings c/f severe lingular pneumonia, intubated. Repeat CT chest showed dense consolidation throughout L lung and R lung base, with cystic changes in the left upper lobe containing patible with necrotizing pneumonia. Bronch on 03/07 completed, no mucous plugs or airway obstruction - thin yellow secretions seen. QUANT gold negative. Ddx is post-viral bacterial (staph/strep) pneumonia, eosinophilic pneumonia, fungal inhalation via vape. Now extubated as of 03/10 and on RA as of 03/11. Infx workup at OSH and RRMC has not yielded pathogenic microorganism.
Dx:
- Bcx (03/07) x2 - NGTD
- Sputum cultures bacterial, fungal (03/07) - Candida likely colonizer
[x] MRSA nares negative
[x] Cocci IgG - negative
[x] Aspergillus - negative
[x] crypto - negative
- Immunoglobulins wnl 03/08
Tx:
- s/p steroids: Hydrocort 50 mg IV q6h (03/07 - 03/11), s/p Solu-Medrol 40 mg IV BID (03/04 - 03/07 am dose)
- Abx:
    - s/p Azithromycin 500 mg IV q24h (03/05 - 3/9)
    - s/p Meropenem 1g q8h (03/07 - 3/13) for 7 day course total
    - s/p Vancomycin (03/05 - 03/09)
    - S/p Zosyn (03/05 - 03/07), Cefepime 2 g IV q12h (03/05), Meropenem 1g IV x1 (03/05), and CTX x1 (03/05)
    - S/p Tamiflu
- Airway clearance: albuterol nebs BID, hypertonic saline BID, IPV, guaifenesin
- CXR today improving compared to previous
- CT Chest 3/26: consolidative changes in the LUL may be cavitation/bronchiectasis, FU with pulm OP, consider bronchoscopy. No increased cough, SOB, no fevers, chills.

#Pleuritic chest pain, stable

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM (continued) Version 1 of 1

Likely 2/2 recovering PNA. EKG with no e/o ischemia/ACS

#Hypokalemia, resolved
With U waves on EKG 3/12. Not on any QT prolonging meds.
-Replete K to 4

#Leukocytosis, improved.
Received steroids, and recovering from infx
- Continue to monitor

Nonischemic cardiomyopathy, query septic , recovered EF
Mitral valve sclerosis, aortic valve thick sclerosis
Initial echo with EF of 25%, diffuse hypokinesis, grade 3 diastolic dysfunction, possible McConnell sign. Mitral valve sclerosis without stenosis.
[x] Repeat TTE - EF 62.5%, < 50% respiratory change although pt was intubated at that time.
- cardiology FU

#Shock liver, resolving
#Mild hepatomegaly
LFTs on arrival elevated, downtrending
- Abd US - mildly enlarged liver, incidental pleural effusion
- Trend LFTs weekly
- Vitamin K 5mg once 03/07

#Acute renal failure, resolved
Noted to have had acute renal failure at OSH, but recovered. Cr here ~0.6.
- BMP daily

#Acute illness/ICU delirium, resolved
No longer having visual hallucinations
- melatonin at bedtime
- trazodone 50 at bedtime prn

Followup
PCP
ENT
Pulm

Follow up requested 3/30

Diet: regular
Activity: as tolerated
Code status: Full Code

Thank you for allowing us to participate in the care of your patient. Please do not hesitate to call or page with questions should they arise.

UCLA Rounder Page/Voicemail (for all nursing issues/questions and otherwise as needed): 310-282-9034

UCLA Hospitalist Administrative Office: 310-319-4698

Physician Progress Notes (last 24 hours) (Notes for yesterday and today) (continued)

Progress Notes by Mary Henary, MD at 3/31/2026 11:31 AM (continued) Version 1 of 1

UCLA Hospitalist Administration Fax: 310-206-3260

AUTHOR:
HENARY, MARY, MD
03/31/26
11:31 AM PDT

CC:
Yue-Shan L Yang, MD

PMD:
No primary care provider on file.
@PCPADD@
None
None

Electronically signed by Mary Henary, MD on 3/31/2026 11:34 AM

Attribution Key

MH.1 - Mary Henary, MD on 3/31/2026 11:31 AM

Wound Progress Notes

Alexandra Zilli, RN (Registered Nurse) 3/19/2026 14:14

Wound Progress Note

Reason for wound Consult: New Admission Skin Assessment

Patient is awake, alert, and oriented

Navroj Aulakh is a 30 y.o. male with the following Problems.

Problem List
Patient Active Problem List
Diagnosis

- Pneumonia

Wound Progress Notes (continued)

Past Medical History:
Past Medical History:

Diagnosis	Date

- Lung disease

Past Surgical History: No past surgical history on file.

Allergies: Patient has no known allergies.

Braden Score: Braden Scale Score: 21

Wound/Ulcer Assessment: Skin intact

Recommendations: Initiate Pressure Prevention and Wound Treatment POC.

3/19/2026, 1406, I met with patient to discuss the wound plan of care including:

- Introduced the patient and family to the wound team members, wound team availability and how to contact the wound care team
- Wound care expectations:
  - How often wound team sees Navroj Aulakh
  - Which provider is managing wounds; provider rounding schedule (if applicable)
- Wound Plan of Care including:
  - List of current, active wounds, etiology and treatment plan
  - Projected healing process and advanced care needs
  - Offered to review the wound photos with them
- Reviewed Prevention Plan of Care
- Addressed questions or concerns

Initiate wound plan of care.

Signature: ZILLI, ALEXANDRA, RN
Date: 3/19/2026
Time: 2:13 PM PDT

Original note by Alexandra Zilli, RN at 3/19/2026 14:14